A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 May 2026

Dr Dejan Fiser
Medical One

Dear Dr Fiser,

Re: Mr Neelkant Vinykant
DOB: 24/11/1978 Phone: 0404217390
Address: 1605/4788 St Kilda Road, MELBOURNE, VIC, 3004
Email: nvinykant@salesforce.com

Summary

- **Improvement in post-fundoplication symptoms including malodorous belching and increased bowel frequency.**
- **Trial of nortriptyline cessation to determine its contribution to symptom control.**
- **Iron studies ordered to assess for iron deficiency; iron infusion to be arranged if required.**
- **No action required by you.**

I had the pleasure of reviewing Neelkant in the clinic today. As you are aware, he has been experiencing significant gastrointestinal symptoms in the context of a previous fundoplication and a period of high stress.

I am pleased to report that Neelkant's symptoms have improved. He previously reported up to five loose bowel movements per day, which has now reduced to two or three formed stools. The malodorous belching that was previously troubling him has also largely resolved. Interestingly, he attributes much of this improvement to the introduction of a kefir. While this specific product contains certain fibres and sweeteners that can often exacerbate bloating and diarrhoea, it appears to be well-tolerated and helpful in his case. He did note that Coca-Cola and avocado seem to trigger the malodorous burping.

Impression and Plan It is difficult to definitively ascertain whether the kefir or the nortriptyline has been the primary driver of his improvement. Neelkant recently ceased his nortriptyline (20 mg nightly), and I suspect this medication may have played a significant role in stabilising his gut function. I have suggested a brief trial period off the medication to see if his symptoms recur. If they do, I have provided him with a new prescription for nortriptyline 25 mg tablets, which may be easier for him to administer than the 10 mg tablets. If the medication is successful, I would aim for a nine-month course to "reset" the gut-brain axis.

Regarding his iron status, his ferritin was noted to be low at 27 ng/mL in December. I have ordered repeat iron studies today. If he remains iron deficient, I have offered to facilitate an iron infusion at St Vincent's. Neelkant will check his private health insurance status regarding any excess before we decide between the private or public system.

From a structural perspective, a previous CT scan was reassuring, excluding pancreatic lesions or nephrolithiasis. I will follow up on his blood results in one week and have asked him to contact my rooms if he does not hear from me. We will continue to review him on an as-needed basis.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

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